A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

28 April 2026

Dr Monika Trivedi

Dear Dr Trivedi,

Re: Ms Victoria Carstairs
DOB: 29/09/1983 Phone: 0423 015 413
Address: Unit 5, 169-173, SHEPPARTON, VIC, 3630
Email: victoria.adams100@yahoo.com
Medicare: 3327445738 / 2

Summary

- **Complex functional gastrointestinal disorder involving suspected carbohydrate malabsorption, visceral hypersensitivity, and pelvic floor dyssynergia.**
- **Arranged for gastroscopy and colonoscopy via the St Vincent's Hospital public waitlist alongside conservative dietary modifications.**

I had the pleasure of conducting a telehealth consultation with Victoria Carstairs.

Symptoms and history of presenting complaint Victoria describes a 20-year history of debilitating gastrointestinal symptoms, which first emerged during a period of significant psychosocial distress in her mid-20s. Her current presentation is characterised by persistent abdominal pain, chronic bloating, and early satiety. She reports that her pain is frequently triggered by stress, caffeine, and dairy, and is notably not relieved by defecation or the passage of wind.

Her bowel habit is erratic. She typically experiences constipation, opening her bowels every three days, which requires significant straining and is associated with a persistent sensation of incomplete evacuation. This alternates with episodes of urgency and diarrhoea. She also reports associated bladder symptoms, including urinary urgency and occasional high-volume voiding. There are no red-flag features such as unintentional weight loss, haematochezia, or melaena.

Diet Victoria's diet is notable for the regular consumption of carbonated soft drinks and frequent use of chewing gum and mints. She previously noted a significant improvement in pain and urgency while adhering to a low FODMAP diet, though she found this difficult to sustain. She currently avoids breakfast and coffee, noting that caffeine appears to exacerbate her discomfort.

Past medical history and medications Victoria's surgical history is significant for a cholecystectomy performed in her 20s. Her medical history includes asthma, which is managed with salbutamol as required; she does not currently use a preventer inhaler or hormonal contraception. She has no other known chronic medical conditions and does not take any regular oral medications.

Social history, family history Victoria lives in Shepparton with her 11-year-old son. Her social history is complex, marked by a previous significant stressors. Regarding her family history, there are no reported instances of inflammatory bowel disease or gastrointestinal malignancy.

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
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Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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Examination As this was a telehealth consultation, I did not perform a physical examination.

Impression and plan Victoria's presentation is suggestive of a multifactorial functional bowel disorder. Firstly, her history of improvement on a low FODMAP diet suggests a component of carbohydrate malabsorption, likely exacerbated by her intake of polyols (sorbitol) in chewing gum and artificial sweeteners in diet soft drinks. Secondly, the chronicity and nature of her pain point towards visceral hypersensitivity. Finally, her symptoms of straining and incomplete evacuation, alongside bladder urgency, are highly suggestive of pelvic floor dyssynergia.

The plan is as follows:

- **Investigations:** I have requested a suite of baseline blood tests. I have also placed her on the waiting list at St Vincent's Hospital for a gastroscopy and colonoscopy to exclude organic pathology, particularly given the duration of her symptoms.
- **Dietary Modifications:** I have advised Victoria to cease using chewing gum and to avoid "diet" or sugar-free soft drinks containing artificial sweeteners. We discussed using regular sugar-sweetened beverages in moderation as an alternative and incorporating two kiwi fruits daily to assist with bowel regularity.
- **Psychosocial Support:** We discussed the significant link between her domestic stress and her gut symptoms.
- **Logistics:** Due to her social situation, we will need to coordinate the logistics of her procedure at St Vincent's carefully, as she requires an adult carer to accompany her and stay with her overnight post-anaesthetic.

Thank you again for referring this patient.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

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